7325 S. Pecos Rd
Suite #102
Las Vegas, NV 89120
Phone: (702) 982-6402
FAX: (702) 202-0674
www.ChamianMedicalGroup.com

CHAMIAN MEDICAL GROUP

Fax

To: Avery Home Health
From: CMG
Fax: (702)899-4479
Pages: 9 pages
Phone:
Date: 6/4/24
Re: Frank Mendoza
cc: DOB: 4/21/1936

☐ Urgent ☑ For Review ☐ Please Comment ☐ Please Reply ☐ Please Recycle

COMMENTS:

* Home Health referral attached w/ clinical notes & demographics *

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Chamian Medical Group, PLLC
7325 S. Pecos Rd Ste 102
Las Vegas, NV 89120-3768

Chart Cover

Print Date : 06/04/2024
Print Time : 02:40 pm
Print User: Enriquez, Fernanda

MENDOZA, FRANK	Account No	MENFRA0001	Date of Birth	04/02/1936
9117 LIVING ROSE ST	Home Phone	(702)263-2153	SSN	XXX-XX-1517
Las Vegas, NV 89123	Cell Phone	(323)383-3232	Gender	M
	Home Fax		DL #	
Provider Chamian, Noel M	Pager		Marital Status	S
Referral N/A	Financial Group	MDC	First Visit Date	07/06/2023

Employment Information

	Patient Position		Office Phone
	e-mail		Office Fax

Guarantor Information

MENDOZA, FRANK	Account No	MENFRA0001	Gender	M
9117 LIVING ROSE ST	Home Phone	(702)263-2153	Date of Birth	04/02/1936
	Home Fax		DL #	
Las Vegas, NV 89123	e-mail		SSN	XXX-XX-1517

Insurance Information

Insurance Company Address	Office Phone City	Code State	Policy Holder Zip	Policy Number Group Number	Copayment	Deductible	% Ins
Medicare P O BOX 6776	(855)609-9960 Fargo	MB ND	MENDOZA, FRANK 58108-6776	9NF5PE1RX51 None	$0.00	$0.00	100%

Health Summary

Notes

CHAMIAN MEDICAL GROUP

7325 S. Pecos Rd. Ste 102 Las Vegas, Nevada 89120
Phone: (702) 982-6402 *Fax: (702) 202-0674*

REFERRAL FOR HOME HEALTH SERVICES

Date: 6/4/2024

Avery Home Health
(F) 702.899.4479

To our valued home health partner,

I would like to refer my patient, FRANK MENDOZA for home health evaluation and treatment services.

The diagnosis for this patient is Age related debility.

Below are my patient's current demographics for your reference:

Full name:	FRANK MENDOZA
DOB:	4/2/1936
Address:	9117 LIVING ROSE ST Las Vegas, NV 89123
Home phone:	(702)263-2153
Cell phone:	(323)383-3232
Primary Insurance:	Medicare
Secondary Insurance:	

Please feel free to contact our office at (702) 982-6402 if you have any questions.

Best Regards,

Noel Chamian, MD

MENDOZA, FRANK 04/02/1936

Default Progress Note
Visit Date: Tue, Apr 30, 2024 9:54 am
Provider: Chamian, Noel M, MD (Supervisor: Chamian, Noel M, MD; Assistant: Salazar, Asencion, MA)
Location: Chamian Medical Group, PLLC

Electronically signed by Noel Chamian, MD on 04/30/2024 10:19:03 AM
Printed on 06/04/2024 at 2:39 pm.

Subjective:

CC: Mr. MENDOZA is a 88 year old male. This is a follow-up visit.

HPI:
Referred by Dr. Ayers

Mr. MENDOZA presents with essential (primary) hypertension. This was first diagnosed several years ago. He is not using any nonpharmacologic treatment modalities. He is tolerating the medication well without side effects. Compliance with treatment has been fair.

Follow up of asthma. It began unknown. Prior work-up has included See Docman for result of the test..

Follow up of pneumoconiosis due to asbestos and other mineral fibers. It began unknown. There are no associated symptoms. Prior work-up has included See Docman for result of the test..

Follow up of age-related physical debility. It began years ago.

ROS:
All other systems were reviewed and nothing pertinent except for the above mentioned symptom/s.

Past Medical History / Family History / Social History:

Last Reviewed on 4/30/2024 10:18 AM by Chamian, Noel M

Past Medical History:
Negative Stress test- 2022
Vertigo
Carpal tunnel
Right leg neuropathy
COVID-19
HTN
Atelectasis
Cervical disc disease
Coronary artery disease
Allergic rhintis
CAD s/p stent
Pneumonia
Right diapragm paralysis

CURRENT MEDICAL PROVIDERS:
Cardiologist: Dr. Ayers/USC
E/N/T: Kong
Neurologist: Dr. CAbahug
None

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MENDOZA, FRANK 04/02/1936
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PREVENTIVE HEALTH MAINTENANCE

PNEUMOCOCCAL VACCINE: was last done 2023

Surgical History:
Disc surgery

Family History:
HTN

Social History:
Occupation:
Retired
Marital Status: Single
Children: 3 children

Tobacco/Alcohol/Supplements:
Last Reviewed on 4/30/2024 10:18 AM by Chamian, Noel M
Tobacco: Current Smoker: He currently smokes every day, (5 pack-year history). Non-drinker

Substance Abuse History:
Last Reviewed on 4/30/2024 10:18 AM by Chamian, Noel M

Mental Health History:
Last Reviewed on 4/30/2024 10:18 AM by Chamian, Noel M

Communicable Diseases (eg STDs):
Last Reviewed on 4/30/2024 10:18 AM by Chamian, Noel M

Immunizations:
Influenza vaccine, quadrivalent, adjuvanted (FLUAD QUAD 2023-2024) 10/11/2023

Allergies:
Last Reviewed on 4/30/2024 10:18 AM by Chamian, Noel M
cetirizine: (Adverse Reaction)
montelukast: (Adverse Reaction)

Current Medications:
Last Reviewed on 4/30/2024 10:18 AM by Chamian, Noel M
scopolamine base 1 mg over 3 days Transdermal Patch,Transdermal 3 day [apply 1 patch by transdermal route to the hairless area behind 1 ear every 72 hours]
meclizine 25 mg oral tablet,chewable [chew 1 tablet (25 mg) by oral route 3 times per day pRN]

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amLODIPine 10 mg oral tablet [take 1 tablet (10 mg) by oral route once daily]
clopidogreL 75 mg oral tablet [take 1 tablet (75 mg) by oral route once daily]
eszopiclone 1 mg oral tablet [take 2 tablets (2 mg) by oral route once daily at bedtime]
atorvastatin 40 mg oral tablet [take 1 tablet (40 mg) by oral route oat night]
aspirin 81 mg oral tablet, delayed release (enteric coated) [take 1 tablet (81 mg) by oral route after breakfast]
albuterol sulfate 90 mcg/actuation Inhalation HFA Aerosol Inhaler [inhale 2 puffs (180 mcg) by inhalation route every 4-6 hours as needed]
ranolazine 500 mg oral Tablet, Extended Release 12 hr [take 1 tablet (500 mg) by oral route 2 times per day]
bisacodyL 5 mg oral tablet, delayed release (enteric coated) [1 tablet once a day prn]
fluticasone propionate 50 mcg/actuation Intranasal Spray, Suspension [2 sprays each nostril at night]
azelastine 137 mcg (0.1 %) Intranasal Aerosol, Spray [spray 2 sprays (274 mcg) in each nostril by intranasal route 2 times per day]

Objective:

Vitals:

Current: 4/30/2024 9:40:55 AM
Ht: 6 ft, 0 in; Wt: 178 lbs; BMI: 24.1T: 97.9 F (temporal); BP: 116/60 mm Hg (left arm, sitting); P: 84 bpm (finger clip, sitting); sCr: 0.8 mg/dL; **GFR: 68.03O2 Sat: 96 % (room air)**Pain Index: **0**

Exams:

GENERAL: well developed, well nourished, in no apparent distress
EYES: lids and conjunctiva are normal; pupils and irises are normal;
E/N/T: normal external ears and nose;; normal external auditory canals and tympanic membranes; Hearing: grossly normal Nasal Septum/Mucosa: turbinates are **markedly swollen** bilaterally; Lips, Teeth and Gums: normal; Oropharynx: normal mucosa, palate, and posterior pharynx;
NECK: Neck is supple with full range of motion; thyroid is normal to palpation;
RESPIRATORY: normal respiratory rate and pattern with no distress; **decreased breath sounds in the RLL; ; expiratory wheezes noted**
CARDIOVASCULAR: normal rate and rhythm without murmurs; normal S1 and S2 heart sounds with no S3, S4, rubs, or clicks; carotids: 2+ amplitude, no bruits; abdominal aorta appears to be of normal size and is without bruits; 2+ pedal pulses; no edema;
GASTROINTESTINAL: no masses palpated; nontender; no organomegaly
LYMPHATIC: no enlargement of cervical nodes; no supraclavicular, suboccipital, periauricular or other nodes;
MUSCULOSKELETAL: digits/nails: no clubbing, cyanosis, or evidence of ischemia or infection; normal gait; grossly normal tone and muscle strength; full, painless range of motion of all major muscle groups and joints no laxity or subluxation of any joints; no masses, effusions, misalignment, crepitus, or tenderness in major joints;
SKIN: no ulcerations, lesions or rashes no skin thickening, induration, or subcutaneous nodules;
NEUROLOGIC: cranial nerves II-XII grossly intact; normal DTR's elicited in biceps, triceps, supinator, knee, and ankle jerk; sensation: normal to touch and pinprick; vibration and proprioception senses intact;
PSYCHIATRIC: mental status: alert and oriented x 3; appropriate affect and demeanor; recent and remote memory are intact; good insight and judgement;

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Assessment:

J45 Asthma
R54 Age-related physical debility
J61 Pneumoconiosis due to asbestos and other mineral fibers
I10 Essential (primary) hypertension

ORDERS:

Lab Orders:

Automated complete blood count with platelets and complete differential (Send-Out)
Comprehensive metabolic panel (Send-Out)
Lipid panel (total cholesterol, HDL, triglycerides) (Send-Out)
Thyroid stimulating hormone (TSH) (Send-Out)
UA, auto, w/micro (Send-Out)
Uric acid (Send-Out)

Procedures Ordered:

Tobacco use assessed (CAD, CAP, COPD, PV)1 (DM)4 (In-House)
Current tobacco non-user (CAD, CAP, COPD, PV)1 (DM)4 (In-House)

Other Orders:

Most recent systolic BP <140 mmHg (Send-Out)
Most recent diastolic BP <90 mmHg (Send-Out)
At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)
At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)
Pain assessment negative using a standardized tool and no follow-up plan needed. (In-House)
At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)
Body Mass Index (BMI), documented (PV) (In-House)
PHQ-9 quick depression assessment (In-House)
Patient is negtive for clinical depression and a follow-up plan is not required (In-House)
Falls risk assessment documented (In-House)
Pt screen for fall risk; document no falls in past year or only 1 fall w/o injury in past year (GER) (In-House)
At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)
Patient encounter documented using a qualified (non-CCHIT) certified EHR (In-House)

Plan:

Asthma

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MEDICATIONS: (see today's med list) At least one Rx was generated and transmitted electronically using SureScripts
RECOMMENDATIONS given include: go to the ER if symptom/s worsen and explained potential side effects of medication/s. Patient refused to have pulmonary consult
FOLLOW-UP: Advised to call if there is no improvement. Instructed to call if new or worsening symptoms develop. Schedule a follow-up visit in 3 months.

Orders:
G8553 At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)

Age-related physical debility

FOLLOW-UP: Schedule a follow-up appointment in 1 week.

Prescriptions: refer to Home health

Pneumoconiosis due to asbestos and other mineral fibersrepeat CT scan and PFT 8/2024
BMI documented PHQ-9 Quick depression assessment done. Patient is NEGATIVE for clinical depression, a follow-up plan not required. No history of fall only 1 fall without injury within the past year.
MEDICATIONS: (see today's med list) At least one Rx was generated and transmitted electronically using SureScripts
RECOMMENDATIONS given include: go to the ER if symptom/s worsen, explained potential side effects of medication/s, and pain assessment negative using a standardized tool and no follow-up plan is needed. Patient refused to have pulmonary cosnult
FOLLOW-UP: Advised to call if there is no improvement. Instructed to call if new or worsening symptoms develop. Schedule a follow-up visit in 3 months.
Smoking Status: Nonsmoker Patient encounter documented using a qualified (non-CCHIT) certified EHR

Orders:
G8731 Pain assessment negative using a standardized tool and no follow-up plan needed. (In-House)
3008F Body Mass Index (BMI), documented (PV) (In-House)
PHQSCR PHQ-9 quick depression assessment (In-House)
G8510 Patient is negtive for clinical depression and a follow-up plan is not required (In-House)
3288F Falls risk assessment documented (In-House)
1101F Pt screen for fall risk; document no falls in past year or only 1 fall w/o injury in past year (GER) (In-House)
G8553 At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)
1000F Tobacco use assessed (CAD, CAP, COPD, PV)1 (DM)4 (In-House)
1036F Current tobacco non-user (CAD, CAP, COPD, PV)1 (DM)4 (In-House)
G8448 Patient encounter documented using a qualified (non-CCHIT) certified EHR (In-House)

Essential (primary) hypertension
Systolic BP Most recent systolic BP <140mmHg Diastolic BP Most recent diastolic BP <90 mmHg
RECOMMENDATIONS given include: exercise, reduction of dietary salt intake, avoid licorice in diet, avoid pseudoephedrine or other stimulants/decongestants in common cold remedies, and perform routine monitoring of blood pressure with home blood pressure cuff.
LAB ORDERS: CBC, comprehensive metabolic panel, lipid panel, TSH, urinalysis, uric acid.

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MEDICATIONS: (see today's med list) At least one Rx was generated and transmitted electronically using SureScripts
FOLLOW-UP: Advised to call if there is no improvement. Schedule a follow-up visit in 3 months.

Orders:
85025 Automated complete blood count with platelets and complete differential (Send-Out)
80053 Comprehensive metabolic panel (Send-Out)
80061 Lipid panel (total cholesterol, HDL, triglycerides) (Send-Out)
84443 Thyroid stimulating hormone (TSH) (Send-Out)
81001 UA, auto, w/micro (Send-Out)
84550 Uric acid (Send-Out)
G8752 Most recent systolic BP <140 mmHg (Send-Out)
G8754 Most recent diastolic BP <90 mmHg (Send-Out)
G8553 At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)

Other Orders
G8553 At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)

Diagnosis and Procedure Summary:

Primary Diagnosis:
J45 Asthma

Orders:
G8553 At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)

R54 Age-related physical debility
J61 Pneumoconiosis due to asbestos and other mineral fibers

Orders:
G8731 Pain assessment negative using a standardized tool and no follow-up plan needed. (In-House)
3008F Body Mass Index (BMI), documented (PV) (In-House)
PHQSCR PHQ-9 quick depression assessment (In-House)
G8510 Patient is negtive for clinical depression and a follow-up plan is not required (In-House)
3288F Falls risk assessment documented (In-House)
1101F Pt screen for fall risk; document no falls in past year or only 1 fall w/o injury in past year (GER) (In-House)
G8553 At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)
1000F Tobacco use assessed (CAD, CAP, COPD, PV)1 (DM)4 (In-House)
1036F Current tobacco non-user (CAD, CAP, COPD, PV)1 (DM)4 (In-House)
G8448 Patient encounter documented using a qualified (non-CCHIT) certified EHR (In-House)

I10 Essential (primary) hypertension

Orders:

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G8553 At least one Rx created at encounter was generated/sent electronically w/ a qualified eRx system (In-House)

Other Orders:

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